



Drug Management Policy

Stelara

Policy Number: 9.129

Version Number: 1

Version Effective Date: 1/1/2025

Impacted Products

- ☐ **All Products**
- ☐ NH Medicaid
- ☐ NH Medicare Advantage
- ☒ NH Clarity plans
- ☐ MA MassHealth ACO
- ☐ MA MassHealth MCO
- ☐ MA Clarity plans/Employer Choice Direct
- ☐ MA Senior Care Options

Benefit

- ☒ Pharmacy benefit
- ☐ Medical benefit

Note: Disclaimer and audit information is located at the end of this document.

Prior Authorization Policy

Products Affected:

- Stelara (ustekinumab)

The Plan may authorize coverage of the above products for members meeting the following criteria:

Covered Use	All FDA approved indications unless otherwise excluded
Required Medical Information	Moderate to severe Plaque Psoriasis (PsO) 1. The member is 6 years of age or older; AND 2. Prescribed by or in consultation with a dermatologist; AND 3. Documentation of one of the following: a. Involvement of at least 3% of total body surface area; OR b. Hands, feet, scalp, face, or genital area affected; AND 4. Documentation of one of the following: a. Inadequate response or adverse reaction to at least a 3 month trial of one biologic DMARD that is FDA-approved for plaque psoriasis; OR b. Inadequate response or adverse reaction to at least a 3 month trial of two conventional therapies (as defined below) in any one of the following combinations (please note: these combinations DO NOT have to be used concurrently): i. 1 topical agent + 1 systemic agent; OR ii. 1 topical agent + 1 phototherapy; OR iii. 1 systemic agent + 1 phototherapy; OR iv. 2 systemic agents; OR a. Contraindication to methotrexate, as determined by the prescriber; AND 5. Documentation of an inadequate response or adverse reaction or contraindication to Enbrel or Humira; AND 6. Documentation that the requested dose will not exceed FDA approved dosing (see Appendix A); AND 7. If the 90 mg syringe is requested, approve if the member meets ONE of the following: a. Member weighs > 100 kg; OR b. Member is currently receiving the 90 mg syringe; OR c. Member has received standard dosing with the 45 mg syringe/vial for at least 3 months with inadequate efficacy Psoriatic Arthritis (PsA); AND 1. The member is 6 years of age or older; AND

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2. Prescribed by in or in consultation with a rheumatologist; **AND**
3. Documentation of One of the following:
 - a. An inadequate response or adverse reaction to at least a 3 month trial of one non-biologic DMARD or contraindication to non-biologic DMARDs; **OR**
 - b. An inadequate response or adverse reaction to at least a 3 month trial of one biologic DMARD that is FDA-approved for psoriatic arthritis; **AND**
4. Documentation of an inadequate response or adverse reaction or contraindication to Enbrel or Humira ; **AND**
5. Documentation that the requested dose will not exceed FDA approved dosing (see Appendix A); **AND**
6. If the 90 mg syringe is requested, approve if the member meets ONE of the following:
 - a. Member weighs > 100 kg; **OR**
 - b. Member is currently receiving the 90 mg syringe; **OR**
 - c. Member has received standard dosing with the 45 mg syringe/vial for at least 3 months with inadequate efficacy

Crohns Disease (CD)/ Ulcerative Colitis (UC)

1. Member is 18 years of age or older; **AND**
2. Prescribed by or in consultation with a gastroenterologist; **AND**
3. Documentation the member has received IV induction dosing within 2 months of initiation therapy with Stelara subcutaneous; **OR**
4. Documentation member is currently stable on Stelara subcutaneous maintenance therapy; **AND**
5. Documented dose will not exceed FDA-approved dosing (see Appendix A)

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Coverage Duration	Initial: 6 months Reauthorization: 12 months
Other criteria	Reauthorization: 1. Initial criteria met; AND 2. One of the following: a. The member has had a positive response to therapy: OR b. Documentation of clinical justification to continue therapy with the requested medication

Appendix A – FDA Approved Dosing

Diagnosis	FDA Approved Dosing
Plaque psoriasis	<u>Adult:</u> <u>Individuals ≥ 18 years of age</u> ≤ 100 kg: SUBQ: 45 mg at 0 and 4 weeks, followed by maintenance of 45 mg every 12 weeks thereafter. > 100 kg: SUBQ: 90 mg at 0 and 4 weeks, followed by maintenance of 90 mg every 12 weeks thereafter. <u>Pediatric:</u> <u>Children ≥ 6 years and Adolescents < 18 years:</u> < 60 kg: SUBQ: 0.75 mg/kg at 0 and 4 weeks, followed by maintenance of 0.75 mg /kg every 12 weeks ≥ 60 to ≤ 100 kg: SUBQ: 45 mg at 0 and 4 weeks, followed by maintenance dose of 45 mg every 12 weeks > 100 kg: SUBQ: 90 mg at 0 and 4 weeks, followed by maintenance dose of 90 mg every 12 weeks

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Psoriatic Arthritis	<u>Adult</u> Initial and maintenance: SUBQ: 45 mg at 0 and 4 weeks, and then every 12 weeks thereafter Coexistent psoriatic arthritis and moderate to severe plaque psoriasis in patients >100 kg: Initial and maintenance: SUBQ 90 mg at 0 and 4 weeks and then 90 mg every 12 weeks thereafter <u>Pediatric</u> <u>Children ≥6 years and Adolescents < 18 years:</u> <60 kg: SUBQ: Initial: 0.75mg /kg at 0 and 4 weeks, followed by maintenance dose of 0.75 mg/kg every 12 weeks ≥60 kg: SUBQ: 45 mg at 0 and 4 weeks, followed by maintenance dose of 45mg every 12 weeks. >100 kg and moderate to severe plaque psoriasis comorbidity: SUBQ: 90 mg at 0 and 4 weeks, followed by maintenance dose of 90 mg every 12 weeks.
Crohn's Disease	<u>Adult</u> Maintenance: SUBQ: 90 mg every 8 weeks after the initial intravenous dose, then 8 weeks thereafter
Ulcerative Colitis	<u>Adult</u> Maintenance: SUBQ 90 mg every 8 weeks after the initial intravenous dose, then 8 weeks thereafter

Applicable Coding

Code	Medication
N/A	

Clinical Background Information and References

1. American Academy of Dermatology Association. Psoriasis Clinical Guideline. <https://www.aad.org/member/clinical-quality/guidelines/psoriasis>. Accessed 2024
2. American Academy of Dermatology Association. Psoriasis Clinical Guideline. <https://www.aad.org/member/clinical-quality/guidelines/psoriasis>. Accessed: October 2024

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3. Athos B., Kaplan J. Management of mild to severe ulcerative colitis in children and adolescents. UpToDate®. July 16, 2024. Accessed: October 2024
4. Cohen R., Stein A. Management of moderate to severe ulcerative colitis in adults. UpToDate®. August 20, 2024. Accessed: October 2024.
5. Elmetts, C. Korman, N. Farley Prater, E. et. al. Joint AAD–NPF Guidelines of care for the management and treatment of psoriasis with topical therapy and alternative medicine modalities for psoriasis severity measures. Journal of the American Association of Dermatology. 84(2):p432-470. Feb. 2021. [https://www.jaad.org/article/S0190-9622\(20\)32288-X/fulltext](https://www.jaad.org/article/S0190-9622(20)32288-X/fulltext)
6. Elmetts, C. Lim, H, Wu, J. et. al. Joint American Academy of Dermatology–National Psoriasis Foundation guidelines of care for the management and treatment of psoriasis with phototherapy. Journal of the American Association of Dermatology. 81(3):p775-804. Sept. 2019. [https://www.jaad.org/article/S0190-9622\(19\)30637-1/fulltext](https://www.jaad.org/article/S0190-9622(19)30637-1/fulltext)
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8. Feuerstein, Joseph D., et al. "AGA Clinical Practice Guidelines on the Medical Management of Moderate to Severe Luminal and Perianal Fistulizing Crohn's Disease." Gastroenterology, vol. 160, no. 7, June 2021, pp. 2496-508, <https://doi.org/https://doi.org/10.1053/j.gastro.2021.04.022>. October 2024.
9. Galdman, D. Ritchlin, C. Treatment of psoriatic arthritis. UpToDate®. September 19, 2023. Accessed: October 2024.
10. Hashash JA, Reguerio M. Medical management of moderate to severe Crohn disease in adults. UpToDate® Last updated September 9, 2024. Accessed October 2024.
11. Menter, A. Strober, B. Kaplan, D. et. al. Joint AAD–NPF guidelines of care for the management and treatment of psoriasis with biologics. Association of Dermatology. 80(4):p1029-1072. April 2019. Accessed October 2024.
12. Reguerio M, Hashash JA. Overview of the medical management of mild (low risk) Crohn disease in adults. UpToDate® available at <https://www.uptodate.com>, accessed October 2024.
13. Stelara prescribing information. Horsham, PA: Janssen Biotech, Inc. Accessed October 2024.

Policy History

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Original Approval Date	Original Effective Date	Policy Owner	Approved by
11/14/2024	1/1/2025	Pharmacy Services	Pharmacy & Therapeutics (P&T) Committee

Policy Revisions History			
Review Date	Summary of Revisions	Revision Effective Date	Approved by

Next Review Date

11/2025

Other Applicable Policies

Reference to Applicable Laws and Regulations, If Any

Disclaimer Information

Medical Policies are the Plan's guidelines for determining the medical necessity of certain services or supplies for purposes of determining coverage. These Policies may also describe when a service or supply is considered experimental or investigational, or cosmetic. In making coverage decisions, the Plan uses these guidelines and other Plan Policies, as well as the Member's benefit document, and when appropriate, coordinates with the Member's health care Providers to consider the individual Member's health care needs.

Plan Policies are developed in accordance with applicable state and federal laws and regulations, and accrediting organization standards (including NCQA). Medical Policies are also developed, as appropriate, with consideration of the medical necessity definitions in various Plan products, review of current literature, consultation with practicing Providers in the Plan's service area who are medical experts in the particular field, and adherence to FDA and other government agency policies. Applicable state or federal mandates, as well as the Member's benefit document, take precedence over these guidelines. Policies are reviewed and updated on an annual basis, or more frequently as needed. Treating providers are solely responsible for the medical advice and treatment of Members.

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The use of this Policy is neither a guarantee of payment nor a final prediction of how a specific claim(s) will be adjudicated. Reimbursement is based on many factors, including member eligibility and benefits on the date of service; medical necessity; utilization management guidelines (when applicable); coordination of benefits; adherence with applicable Plan policies and procedures; clinical coding criteria; claim editing logic; and the applicable Plan – Provider agreement.

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